

Cardiac Arrest Post-Resuscitation Care

Indications/Inclusion Criteria:

- Induced therapeutic hypothermia is indicated in cardiac arrest patients after return of spontaneous circulation (ROSC) is achieved. The **Hospital Environment Goal Temp 32° to 34° C (89.6° F)**
- The presenting rhythm does not matter – all patients in cardiac arrest who do not meet exclusion criteria are eligible regardless of initial rhythm (VF/VT/PEA/Asystole)

Exclusion Criteria for Therapeutic Hypothermia:

- Traumatic cause for cardiac arrest
- Time of pulselessness < 5 minutes
- Purposeful movement of extremities
- **Infeasibility of initiating protocol in the environment where the patient has achieved ROSC**

EMS COLD IV Saline Bolus Hypothermia initiation is NO LONGER RECOMMENDED

EMT/AEMT

- Remove patient's clothing
- Apply ice packs (or cold/wet clothing/towels) to groin, axilla, head, and neck (if patient is not already hypothermic)
- Maintain SpO2 >94%, avoid hyperventilation

EMT/AEMT - Expanded Scope

- **NS/LR 500ml IO Bolus x 1**
- Elevate Head to 30 degrees (if feasible)

Paramedic

- If patient has SBP > 110 mmHg **ELEVATE HEAD 30 degrees** (if feasible)
- Ventilate to Goal O2 Saturation >94 %
- Obtain 12-Lead (if available)
- If hypotensive after ROSC, titrate to goal SBP > 110 mmHg
 - **NS/LR** wide open
 - **Epinephrine Push Dose** pressor (1:100,000) 10-20 mcg (1-2 mL) IO/IV every 2-3 minutes PRN (max total 100 mcg / 10 mL) to maintain SBP > 110 or improve signs/symptoms of shock
- **Amiodarone** 150 mg IO/IV over 10 minutes – IF patient had at least one episode of VF or VT and did not receive amiodarone or full dose of amiodarone during resuscitation